

MRN: [REDACTED]

UID: F6CFE506-168A-475A-AB1F-1AC4B2EFE24A
TCGA-AC-A2FB-01A-PR

Redacted

Age: [REDACTED] Sex: [REDACTED]
DOB: [REDACTED]



Surgical Pathology

Criteria	Yes	No
Diagnosis Discrepancy		X
Primary Tumor Site Discrepancy		X
HIPAA Discrepancy		X
Prior Malignancy History		X
Dual/Synchronous Primary Cancer		X
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	W 6/10/11	W 6/10/11
Date Reviewed	6/10/11	6/10/11

Histopathological Examination

Path#: [REDACTED]

Collected:

Received:

1 Complete:

Pre-Op Diagnosis : Absence Of Breast, Personal History Of Breast Cancer - [REDACTED]
Order Physician : MD
Specimens : Breast, left simple mastectomy for expo study. Suture marking margins: long-lateral, short-superior
Lymph Node, Sentinel left axillary

Frozen Diagnosis :

Report

: GROSS EXAMINATION:

A. The specimen is received in a container labeled with the name of the patient and identified as breast, left simple mastectomy. The specimen consists of a mastectomy measuring en bloc 11.5 x 9.5 x 4 cm and weighing 158 grams. There is an anterior ellipse of skin measuring 6 x 3 cm with a central 3 cm areolar complex with retracted nipple. Sections of nipple/areola are sampled in block 1. The skin is sampled in block 2. The superficial superior margin is inked orange and the superficial inferior margin is inked blue. The deep margin is inked green and the specimen sectioned, showing an oval grey-tan firm mass measuring 2.8 cm in the upper inner quadrant of the breast. Grossly, this mass is 0.4 cm from the inked deep margin of resection, which is submitted in block 3. Sections of the mass are submitted in blocks 4-7. The remaining breast is composed of yellow lobulated adipose tissue admixed with islands of gray-white rubbery tissue. The breast is divided into four quadrants, upper inner, lower inner, upper outer and lower outer and sampled in blocks 8-11 respectively. Minimal axillary tissue is present and no lymph nodes are identified. Formalin fixation time is 11 hours.

B. The specimen is received in a container labeled with the name of the patient and identified as lymph axillary sentinel node.

Specimen Size: 1.8 x 1.5 x 1 cm

No. of Nodes: 1

Size of Nodes: 1.8 cm

The specimen is sectioned and entirely submitted.

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100-0-3
carcinoma, infiltrating lobular, NOS 8520/3
Site: breast, NOS 8520.9
6/10/11

DIAGNOSIS BASED ON GROSS AND MICROSCOPIC EXAMINATION:

A. Breast, left, simple mastectomy:
- Invasive lobular adenocarcinoma, pleomorphic variant, Nottingham grade 2, 2.8 cm (see cancer case summary checklist below).

B. Lymph node, left axillary sentinel, excision:
- One lymph node, no tumor present (0/1).

MACROSCOPIC

SPECIMEN TYPE: Mastectomy.
LYMPH NODE SAMPLING: Sentinel lymph node only.
LATERALITY: Left.
TUMOR SITE: Upper inner quadrant.

MICROSCOPIC

SIZE OF INVASIVE COMPONENT:
Greatest dimension: 2.8 cm (based on gross tumor size).
HISTOLOGIC TYPE: Invasive lobular adenocarcinoma, pleomorphic variant.

HISTOLOGIC GRADE:

NOTTINGHAM HISTOLOGIC SCORE

Tubule formation: Minimal less than 10% (score=3)
Nuclear pleomorphism: Moderate increase in size, ect. (score=2)
Mitotic count for a 40 x objective with a field area of 0.152 mm²: 0-5 mitoses per 10 HPF (score=1)
Total Nottingham Score: Grade 2 (6-7 points)

EXTENT OF INVASION

PRIMARY TUMOR (PT): pT2: Tumor more than 2.0 but not more than 5.0 cm in greatest dimension.
REGIONAL LYMPH NODES (PN): pN0: No regional lymph node metastasis histologically (i.e. none greater than 0.2 mm), no additional examination for isolated tumor cells.
Number examined: 1
Number involved 0
DISTANT METASTASIS (PM): pMX: Cannot be assessed.

MARGINS: Uninvolved by invasive carcinoma.
Distance of carcinoma from closest margin: 4 mm (deep margin).

VENOUS/LYMPHATIC (LARGE/SMALL VESSEL) INVASION (V/L): Absent.

MICROCALCIFICATIONS: Present in non-neoplastic tissue.
ADDITIONAL PATHOLOGIC FINDINGS:

Proliferative fibrocystic changes with ductal hyperplasia, columnar cell change, sclerosing adenosis (confirmed by presence of p63 positive myoepithelial cells surrounding epithelial structures, with appropriate controls), apocrine metaplasia, and associated microcalcifications.

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Prior biopsy site.

COMMENT:

Estrogen and progesterone receptors and Her2neu studies were performed on the prior biopsy with results of estrogen receptor 2+, progesterone receptor 3+, and HER2/neu 3+. E cadherin, also performed on the prior biopsy, was negative, confirming a lobular phenotype.

Intradepartmental consultation obtained.

Results communicated to

Pathologist

Electronically Signed by:

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